REXOM™ FOLIC ACID TABLET 5MG

Name and Strength of Active Substances
Each tablet contains:
Folic acid ………………………….. 5mg

Product Description
Yellow colour, round with top-score tablet

Pharmacodynamics
Folic acid is a member of the vitamin B group which is reduced in the body to tetrahydrofolate, a co-enzyme active in several metabolic processes and produces a haemopoietic response in nutritional megaloblastic anaemias. Folic acid is rapidly absorbed and widely distributed in body tissues.

Folic acid is used in the treatment and prevention of folate deficiency states.

Pharmacokinetics
Absorption
Folic acid is rapidly absorbed from the gastrointestinal tract, mainly from the proximal part of the small intestine. Dietary folates are stated to have about half the bioavailability of crystalline folic acid. The naturally occurring folate polyglutamates are largely deconjugated and reduced by dihydrofolate reductase in the intestine to form 5-methyltetrahydrofolate (5MTHF). Folic acid given therapeutically enters the portal circulation largely unchanged, since it is a poor substrate for reduction by dihydrofolate reductases.

Distribution
Via portal circulation. 5MTHF from naturally occurring folate is extensively plasma bound. The principal storage site of folate is in the liver; it is also actively concentrated in the CSF. Folate is distributed into breast milk.

Metabolism
Therapeutically given folic acid is converted into the metabolically active form 5MTHF in the plasma and liver. There is an enterohepatic circulation for folate.

Elimination
Folate metabolites are eliminated in the urine and folate in excess of body requirements is excreted unchanged in the urine. Folic acid is removed by haemodialysis.

Indications
For the prevention and treatment of folate deficiency states.
For the prevention of neural tube defect in the foetus.

Recommended Dose
For the prevention and treatment of folate deficiency states.
Adult:
Initially 10mg-20mg mg daily for 14 days or until haematopoietic response obtained.
Daily maintenance dosage: 2.5mg-10mg.

Child up to 1 year	: 250 mcg/kg daily
1 - 5 years	: 2.5mg/day
6-12 years	: 5mg/day

For prevention of neural tube defect in the foetus - 5 mg daily starting before pregnancy and continued through the first trimester.

Route of Administration
Oral

Contraindications

- Hypersensitivity to folic acid or to any of the excipients.
- Long-term folate therapy is contraindicated in any patient with untreated cobalamin deficiency. This can be untreated pernicious anaemia or other cause of cobalamin deficiency, including lifelong vegetarians. In elderly people, a cobalamin absorption test should be done before long-term folate therapy. Folate given to such patients for 3 months or longer has precipitated cobalamin neuropathy. No harm results from short courses of folate.
- Folic acid should never be given alone in the treatment of Addisonian pernicious anaemia and other vitamin B_{12} deficiency states because it may precipitate the onset of subacute combined degeneration of the spinal cord.
- Folic acid should not be used in malignant disease unless megaloblastic anaemia owing to folate deficiency is an important complication.

Warning and Precautions

- Patients with vitamin B_{12} deficiency should not be treated with folic acid unless administered with adequate amounts of hydroxocobalamin, as it can mask the condition but the subacute irreversible damage to the nervous system will continue. The deficiency can be due to undiagnosed megaloblastic anaemia including in infancy, pernicious anaemia or macrocytic anaemia of unknown aethiology or other cause of cobalamin deficiency, including lifelong vegetarians.
- Caution should be exercised when administering folic acid to patients who may have folate dependent tumours.
- This product is not intended for healthy pregnant women where lower doses are recommended, but for pregnant women with folic acid deficiency or women at risk for the reoccurrence of neural tube defects.
- This product contains lactose monohydrate. Patients with rare hereditary problems of galactose intolerance, total lactase deficiency or glucose – galactose malabsorption should not take this medicine.

Interactions with Other Medicaments

- There is a specific interaction between phenytoin and folate such that chronic phenytoin use produces folate deficiency. Correction of the folate deficiency reduces plasma phenytoin with potential loss of seizure control. Similar but less marked relationship exist with all anti-convulsant treatments including sodium valproate, carbamazepine and the barbiturates (including phenobarbital and primidone). Sulphasalazine and triamterene also inhibit absorption.
- Antibacterials – chloramphenicol and co-trimoxazole may interfere with folate metabolism.
- Folic acid may interfere with the toxic and therapeutic effects of methotrexate. Methotrexate and trimethoprim are specific anti-folates and the folate deficiency caused by their prolonged use cannot be treated by Rexom$_{TM}$ Folic Acid.
- Folate supplements enhance the efficacy of lithium therapy.
- Folinic acid should be used.
- Nitrous oxide anaesthesia may cause an acute folic acid deficiency.
- Both ethanol and aspirin increase folic elimination.

Pregnancy & Lactation
Pregnancy
There are no known hazards to the use of folic acid in pregnancy, supplements of folic acid are often beneficial.

Non-drug - induced folic acid deficiency, or abnormal folate metabolism, is related to the occurrence of birth defects and some neural tube defects. Interference with folic acid metabolism or folate deficiency induced by drugs such as anticonvulsants and some antineoplastics early in pregnancy results in congenital anomalies. Lack of the vitamin or its metabolites may also be responsible for some cases of spontaneous abortion and intrauterine growth retardation.

Lactation

Folic acid is actively excreted in human breast milk. Accumulation of folate in milk takes precedence over maternal folate needs. Levels of folic acid are relatively low in colostrum but as lactation proceeds, concentrations of the vitamin rise. No adverse effects have been observed in breast fed infants whose mothers were receiving folic acid.

Adverse Effects / Undesirable Effects

The rare reported side effects associated with the use of folic acid involve the gastrointestinal disorders are such as anorexia, nausea, abdominal distension and flatulence and immune system disorders such as allergic reactions, comprising erythema, rash, pruritus, urticaria, dyspnoea, and anaphylactic reactions (including shock).

Overdose and Treatment

No special procedures or antidote are likely to be needed.

Effects on ability to drive and use machines

No effect on concentration and co-ordination.

Storage Conditions

Store below 30ºC. Keep container tightly closed. Protect from heat and light.

Dosage Forms and Packaging Available

REXOM™ FOLIC ACID TABLET 5MG is packed in Alu-PVDC blister pack. Each blister are packed into 10 x 10's and 100 x 10's, in a carton along with a package insert.

REXOM™ FOLIC ACID TABLET 5MG is also packed in 100's per container.

For export only, REXOM™ FOLIC ACID TABLET 5MG is also packed in 500's per container.

Registration Number

MAL19911375XZ

Manufactured by & Product Registration Holder:

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